Responsible AI Review Checklist

Copilot Output Review for Clinical Use — Riverbend Cardiology Associates

Document Overview

Prepared by	Health Administration Coordinator, for the Compliance Officer. Submitted for approval as the clinic's standard responsible AI review process for Copilot tools, ahead of the internal compliance review in six weeks.
Scope	Dragon Copilot (encounter documentation drafts, coding suggestions) and Microsoft 365 Copilot (patient communications, care-coordination summaries): the four output types in Scenario Package Samples A–D and Workflow Sections 1.1–1.2.
How to use	Before any Copilot output is saved, sent or entered into the record, the responsible person completes Checkpoints 1–4 in Component 3 for that output type, using the risk findings in Component 2. **Stop rule:** an output with an open PRESENT finding or an incomplete checkpoint is not signed, billed or sent.
Review log	**AI Output Review Log:** one line per released output, kept by the health administration coordinator. Entries use the encounter or account number and record the reviewer, date, checkpoint results and issues found. No clinical narrative goes in the log.
Data notice	Fictional training scenario. No real protected health information appears in this document.

Component 1—Integration Point and Information Flow Summary

Flow stages use the Scenario Package definitions (Section 3.1): Generation → Human Review → Integration / Transmission → Downstream Use. Risks are flagged explicitly as GAP, HANDOFF RISK, CASCADE RISK or RISK.

Output Type	Info Category (Structured / Unstructured)	Integration Point	Downstream Destination	Flow Stage	Flow Gaps / Handoff Risks
A. Encounter documentation draft (HPI, assessment, plan) Dragon Copilot Ref: Sample A; 1.1 Steps 1, 3, 4	**Unstructured** Free-text narrative drafted from ambient audio. No field-level validation, so errors and unconfirmed conclusions are only caught by reading the text.	EHR documentation module. Dragon drafts the note directly in the encounter during the visit (ambient capture, with patient consent).	Legal medical record (chart) once the provider signs. Also the source content for outputs B, C and D.	**Current:** Generation (draft, not signed). **Path:** Generation → Human Review (provider) → Integration (signature into chart) → Downstream Use. **Review gate:** before signature.	**GAP —** Provider review before signing is inconsistent and undocumented; nothing shows the AI assessment was checked. **HANDOFF RISK —** On signature, the unconfirmed AI assessment ("cardiac etiology is unlikely") becomes part of the legal record; fixing it later needs a formal addendum. **CASCADE RISK —** The note feeds B, C and D, so one error here is repeated in three other outputs.

Output Type	Info Category (Structured / Unstructured)	Integration Point	Downstream Destination	Flow Stage	Flow Gaps / Handoff Risks
B. Coding suggestion (ICD-10 R07.89; E&M 99214) Dragon Copilot Ref: Sample B; 1.1 Steps 2, 4	**Structured** Discrete, codified fields, not free text. Codes look final and machine-ready, so they can move on without anyone reading them.	EHR coding/billing module. Auto-populated to the coding queue, pending provider acceptance.	Billing system / claims queue. The biller submits the claim to the payer.	**Current:** Human Review (pending provider acceptance), but already sitting in the queue. **Path:** Generation → Human Review (acceptance) → Integration (automatic flow to claims queue) → Downstream Use (claim submitted). **Review gate:** before acceptance; second check before claim submission.	**GAP —** Codes can auto-populate the queue with no separate confirmation record of who accepted which code, and when. **HANDOFF RISK —** The provider-to-billing handoff is automatic: no person stands between acceptance and the claims queue. **RISK —** Codes are derived from the unsigned draft (A). If the assessment changes, the codes may not. 99214 is only “medium” confidence.
C. Patient communication draft (visit-summary email, appointment reminder) Microsoft 365 Copilot (Outlook) Ref: Sample C; 1.2 Steps 1–4	**Unstructured** Free-text, patient-facing email. Name and visit details are auto-filled from a structured mail-merge list.	Outlook draft (M365 Copilot). Recipient and merge fields filled from the mail-merge list.	**External:** the patient, by patient portal message or direct email. The patient acts on the instructions.	**Current:** Human Review (awaiting coordinator send). **Path:** Generation → Human Review (coordinator) → Transmission (portal / email) → Downstream Use (patient acts). **Review gate:** before Send.	**GAP —** No standard pre-send review step or sign-off log. **HANDOFF RISK —** A clinical conclusion moves from the provider’s note into a coordinator’s email before the provider has confirmed it (“determined it is not related to your heart”). **RISK —** The mail-merge has already matched a recipient’s name to another patient’s visit date in a prior batch: a wrong-patient disclosure path. **RISK —** Once sent, the message cannot be recalled.
D. Care-coordination summary (referral note, prior-authorization follow-up) Microsoft 365 Copilot (Word / Teams) Ref: Sample D; 1.2 Steps 1–4	**Unstructured** Narrative clinical summary with patient history, written for a reader outside the clinic.	Word / Teams draft (M365 Copilot), then secure email or upload to the shared coordination drive.	**External:** the referral partner’s inbox and/or the shared coordination drive. The partner schedules the stress-test follow-up.	**Current:** Human Review, ready for Transmission. **Path:** Generation → Human Review (coordinator) → Transmission (secure email / drive) → Downstream Use (partner schedules). **Review gate:** before send or upload.	**GAP —** Clinical detail leaves the clinic with no disclosure-minimization check. **HANDOFF RISK —** The summary presents the unconfirmed AI assessment as the provider’s view (“low suspicion for cardiac cause per treating provider”), and the partner may triage on it. **RISK —** That wording conflicts with the stress-test referral and could lead the partner to deprioritize scheduling. **RISK —** Two exit channels (email or drive), no single record of what was sent, and drive access is not defined.

Component 2—Responsible AI Boundary and Risk Review

Boundary rule: Copilot may draft. Only the treating provider may make a clinical assessment, accept codes for billing, or approve clinical statements sent to patients or partners. Only the assigned staff member may release an output.

Rating key: PRESENT = the risk is in the sample; fix before release. REQUIRES REVIEW = the risk depends on facts the reviewer must check and record. ABSENT = no specific risk in this sample; standard checkpoints apply.

Output Type	Privacy Risk (Present / Absent / Requires Review + note)	Unsupported Inference (Present / Absent / Requires Review + note)	Inappropriate Disclosure (Present / Absent / Requires Review + note)
A. Encounter documentation draft Dragon Copilot · Sample A	**REQUIRES REVIEW** The draft holds full PHI (identity, symptoms, plan) but stays inside the EHR, which is the correct system. Ambient capture can pick up remarks by family members or personal details that do not belong in the chart. **Check:** correct patient and encounter; ambient-capture consent documented; non-clinical content removed.	**PRESENT** “Symptoms are most consistent with musculoskeletal chest wall pain; cardiac etiology is unlikely” is a diagnostic conclusion written by the AI and not yet confirmed by the provider. Only the provider may make an assessment. HPI wording (“non-exertional,” “resolved without intervention”) must also match what the patient actually said. **Action:** provider confirms, edits or deletes the assessment line before signing.	**ABSENT** Nothing is transmitted at this stage. The draft stays inside the EHR under normal access controls, and saving to the chart is its intended use. Its disclosure risk arises only when its content is reused in C and D, and is controlled at those outputs’ checkpoints. **Check:** none beyond standard sign-off.
B. Coding suggestion Dragon Copilot · Sample B	**ABSENT** The codes are linked to the patient, but they move only inside the clinic’s billing module for a permitted payment purpose, and the link to the Sample A encounter is automatic. No new recipient is introduced. **Check:** covered by the encounter match in Checkpoint 1.	**REQUIRES REVIEW** Proposing codes is the tool’s intended function, but both codes rest on the unconfirmed assessment in A. R07.89 (Other chest pain) aligns with the AI’s chest-wall conclusion, and 99214 (moderate complexity) is flagged only “medium” confidence. A confidence label is not authorization. **Action:** provider checks each code against the signed assessment; 99214 must be supported by documented decision-making.	**REQUIRES REVIEW** Accepted codes flow automatically to the claims queue and reach the payer on the claim. Billing disclosure is permitted, but an unverified diagnosis code would send inaccurate health information to the payer, where it is hard to correct, and no confirmation record exists. **Check:** biller confirms a signed note and an acceptance record before submission.
C. Patient communication draft M365 Copilot (Outlook) · Sample C	**PRESENT** The email is built on patient identifiers (name, visit details) filled from a mail-merge list that has already paired one patient’s name with another patient’s visit date. Direct email is less controlled than the portal. **Action:** verify every merge field against the record using two identifiers; portal by default; no batch sends of visit summaries until the merge fault is fixed.	**PRESENT** “Dr. Lee reviewed your chest discomfort and determined it is not related to your heart” restates the unconfirmed AI assessment, strengthens it (“unlikely” becomes “not related”), and attributes it to Dr. Lee. A patient reassured this way may ignore recurring symptoms. **Action:** remove it or route it to the provider; only provider-approved clinical wording may be sent.	**PRESENT** Sending would disclose an unconfirmed clinical conclusion to the patient as fact and, given the merge fault, possibly to the wrong patient. Once sent outside the clinic, it cannot be recalled. **Action:** do not send until provider approval and the recipient check are recorded.

Output Type	Privacy Risk (Present / Absent / Requires Review + note)	Unsupported Inference (Present / Absent / Requires Review + note)	Inappropriate Disclosure (Present / Absent / Requires Review + note)
D. Care-coordination summary M365 Copilot (Word / Teams) · Sample D	**REQUIRES REVIEW** Patient identity and history (hyperlipidemia, family history of CAD) leave the clinic. Secure email is an appropriate channel, but the partner's address must be verified, and access to the shared coordination drive is not defined. **Check:** verified partner address; drive folder restricted to named users.	**PRESENT** "Low suspicion for cardiac cause per treating provider" presents the AI's unconfirmed assessment as the provider's opinion. It also conflicts with the reason for referral (stress-test follow-up), which could lead the partner to deprioritize scheduling. **Action:** provider confirms the line or it is removed; the referral reason must match the order.	**REQUIRES REVIEW** Sharing with a referral partner for treatment is a legitimate purpose, and the history listed is relevant to a stress test, so the disclosure is not inappropriate in itself. But there is no minimization check, an unconfirmed conclusion would be disclosed as fact, and a drive upload may expose the summary beyond the intended partner. **Check:** minimization check: include only what the partner needs for this referral.

Component 3—Verification Checkpoint Sequence

The four checkpoints follow the information-flow stages in Scenario Package Section 3.1, so every output type passes through the same sequence and each stage has a named owner and a record. Complete them in order; if a checkpoint fails, the output stops there.

Checkpoint	Flow stage	Question the reviewer answers
CP1 — Source & Accuracy Verification	Generation → Human Review	Is the content correct, and is it attached to the right patient, encounter and recipient?
CP2 — Boundary & Risk Check	Human Review	Does the output cross a responsible AI boundary? Resolve every Component 2 finding.
CP3 — Authorization & Release	Integration / Transmission	Has the accountable person approved it, and is it going to the right system or recipient by the right channel?
CP4 — Post-Release Audit & Correction	Downstream Use	Did it arrive and get used correctly, and are errors found, corrected and reported?

Checkpoint sequence by output type

Output Type	Checkpoint	Stewardship Criteria	Responsible Role	Documentation Requirement
A. Encounter documentation draft Dragon Copilot	**CP1** Source & Accuracy Verification	Draft is in the correct patient's chart and encounter. Ambient-capture consent is documented. HPI matches what the patient reported (onset two weeks ago; brief; non-exertional; resolved). Plan reflects what was discussed (continue medications; 3-month follow-up).	Treating provider	Consent recorded in the encounter. Provider edits kept in the EHR audit trail.
A. Encounter documentation draft Dragon Copilot	**CP2** Boundary & Risk Check	Apply Component 2 (A). Confirm, edit or delete the AI assessment "cardiac etiology is unlikely": no diagnosis may remain that the provider did not make. Remove non-clinical or third-party remarks picked up by ambient capture.	Treating provider	Attestation line in the note: "AI-assisted draft; assessment reviewed and confirmed/edited by [provider], [date]."
A. Encounter documentation draft Dragon Copilot	**CP3** Authorization & Release	Provider signs only after CP1 and CP2 are complete. Outputs C and D may not be released until this note is signed.	Treating provider (signs). Health administration coordinator confirms signed status before C or D is released.	Electronic signature and timestamp, with the attestation present. Signed status noted in the log entries for C and D.

Output Type	Checkpoint	Stewardship Criteria	Responsible Role	Documentation Requirement
A. Encounter documentation draft Dragon Copilot	**CP4** Post-Release Audit & Correction	Monthly random sample of signed AI-assisted notes (suggested: 3 per provider): attestation present; assessment shows provider review. Errors found after signing are corrected by addendum, never overwritten, and traced to any B, C or D output that used them.	Health administration coordinator (audit). Treating provider (addendum). Compliance officer (reviews trends).	Monthly audit entry in the AI Output Review Log; EHR addendum record; summary to the compliance officer.
B. Coding suggestion Dragon Copilot	**CP1** Source & Accuracy Verification	Codes are attached to the correct encounter (Sample A). R07.89 matches the diagnosis in the provider-confirmed assessment, not the AI draft. If the provider changed the assessment at A-CP2, the codes are re-checked.	Treating provider	Accept, edit or reject action for each code recorded in the coding module.
B. Coding suggestion Dragon Copilot	**CP2** Boundary & Risk Check	Apply Component 2 (B). Treat “high/medium” confidence as advisory only. Accept 99214 only if the signed note documents moderate-complexity decision-making; otherwise the provider selects the supported level. No code may encode an unconfirmed diagnosis.	Treating provider	Acceptance record showing who accepted each code and when; reason recorded if the E&M level is changed.
B. Coding suggestion Dragon Copilot	**CP3** Authorization & Release	A claim moves forward only when (1) the note is signed and (2) every code has an acceptance record. Until the EHR is set to hold unaccepted codes, the biller holds any claim missing either item.	Biller (pre-submission check). Health administration coordinator requests the EHR hold setting.	Pre-submission check recorded in the billing system or log: signed note confirmed; acceptance record confirmed.
B. Coding suggestion Dragon Copilot	**CP4** Post-Release Audit & Correction	Monthly comparison of AI-suggested and accepted codes, and of the E&M level mix, watching for drift to higher levels. Denials are traced to their source. Errors are fixed with a corrected claim.	Biller with the health administration coordinator. Compliance officer reviews.	Monthly coding audit report; corrected-claim records; findings in the AI Output Review Log.
C. Patient communication draft M365 Copilot (Outlook)	**CP1** Source & Accuracy Verification	Every merge field (name, visit date, provider, instructions) matches this patient’s record, checked with two identifiers. Content matches the signed note, not the draft. No placeholder such as “[Patient Name]” remains.	Front-desk coordinator	Log entry: reviewer initials, identifiers matched, source = signed note.
C. Patient communication draft M365 Copilot (Outlook)	**CP2** Boundary & Risk Check	Apply Component 2 (C). Remove, or route to the provider, any diagnosis or interpretation, e.g. “determined it is not related to your heart.” Wording must not overstate the provider’s assessment. Include the plan’s safety-net instruction (contact the clinic if symptoms recur).	Front-desk coordinator (screens). Treating provider (approves clinical wording).	Provider approval of the clinical wording (EHR message or Teams reply), referenced in the log.
C. Patient communication draft M365 Copilot (Outlook)	**CP3** Authorization & Release	Send through the patient portal by default; direct email only if the patient has agreed to email. Send visit summaries one at a time, with no batch merges until the merge fault is fixed. Do not send while any PRESENT finding is open.	Front-desk coordinator	Sent item retained. Log records channel, recipient check and provider-approval reference.

Output Type	Checkpoint	Stewardship Criteria	Responsible Role	Documentation Requirement
C. Patient communication draft M365 Copilot (Outlook)	**CP4** Post-Release Audit & Correction	Weekly spot-check of sent AI-drafted messages (suggested: 10%). A message sent to the wrong patient or with incorrect clinical content is reported the same day for privacy-incident assessment, and a correction is sent.	Health administration coordinator (spot-check). Compliance officer (incident assessment).	Spot-check results in the log; incident report and correction record when applicable.
D. Care-coordination summary M365 Copilot (Word / Teams)	**CP1** Source & Accuracy Verification	Correct patient and correct referral partner. Content matches the signed note and the referral order. The reason for referral (stress-test follow-up) is consistent with the clinical statements in the summary.	Front-desk coordinator	Log entry: identifiers matched, source = signed note and referral order.
D. Care-coordination summary M365 Copilot (Word / Teams)	**CP2** Boundary & Risk Check	Apply Component 2 (D). “Low suspicion for cardiac cause per treating provider” is confirmed by the provider or removed. Minimization: keep only history the partner needs (hyperlipidemia and family history of CAD are relevant to a stress test) and remove anything else.	Treating provider (clinical statements). Front-desk coordinator (minimization).	Minimization check and provider confirmation recorded in the log.
D. Care-coordination summary M365 Copilot (Word / Teams)	**CP3** Authorization & Release	Send by secure email to the partner address in the clinic’s referral directory, or upload to a drive folder limited to named users. Use one channel per referral so there is a single record.	Front-desk coordinator. Health administration coordinator maintains the directory and drive permissions.	Secure-email sent record or drive upload record; channel and recipient logged.
D. Care-coordination summary M365 Copilot (Word / Teams)	**CP4** Post-Release Audit & Correction	Confirm the partner received the summary and scheduled the test. If the summary contained an error, send a correction and log it. Review shared-drive access quarterly and remove files past retention.	Health administration coordinator. Compliance officer (quarterly access review).	Receipt or scheduling confirmation; correction record; quarterly drive-access review record.

Component 4—Implementation Rationale (400–600 words)

Governance position. Riverbend's position is that Copilot drafts and people decide: no AI-generated note, code, patient message or referral summary is signed, billed or sent until the accountable team member has verified it and recorded that verification.

How the components work together. Component 1 maps where each of the four outputs enters and leaves clinic systems, and it shows that every gap sits at the same handoff: Human Review to Integration or Transmission, where no review is recorded today. Component 2 tests each output at that handoff against three boundaries, and the ratings are deliberately uneven. Coding suggestions (B) are rated Absent for privacy because they never leave the billing module, while the patient email (C) is rated Present on all three risks. Component 3 then turns each finding into a checkpoint with a named owner and a record. Its four checkpoints follow the scenario's flow stages, so staff apply one sequence whether they are signing a note or sending an email, and the compliance officer can audit each stage from the AI Output Review Log.

Highest-priority review steps. The top priority is Checkpoint 2 for the encounter note (A). The AI-drafted line "cardiac etiology is unlikely" is the source of every unsupported-inference finding in the workflow: R07.89 aligns with it (B), the patient email hardens it into "not related to your heart" (C), and the referral attributes it to the treating provider (D). Confirming or correcting that one line before signature stops the error at its source. Second is Checkpoint 3 for the patient communication (C), because it goes straight to the patient, cannot be recalled, and comes from a mail-merge that has already mismatched patients. Portal-first, one-at-a-time sending and provider approval of clinical wording are mandatory until that fault is fixed. Third is Checkpoint 3 for coding (B): accepted codes flow to claims automatically, so the biller's hold on claims without an acceptance record closes the gap noted in the systems reference.

Compliance and stewardship considerations. First, signing an AI-assisted note makes the provider accountable for every line; the attestation records that review took place, and later corrections must be made by addendum. Second, confidence labels are not authorization: an E&M level such as 99214 must be supported by documented decision-making, or the clinic faces billing compliance exposure. Third, referral disclosures for treatment are generally permitted, but the clinic should still apply the minimization check for D and restrict the shared coordination drive to named users. Fourth, the review log must itself protect privacy by recording encounter or account numbers rather than clinical narrative. Fifth, the checklist must fit the working day: provider steps are built into note signing rather than added as a separate task, because time pressure is why review is skipped today. Before adoption, all eight providers and both front-desk coordinators need a short briefing on the boundary rule, and the checklist should be revisited whenever Copilot features or output types change. The first monthly audit, reported to the compliance officer, will give the clinic documented evidence of a working process ahead of the compliance review in six weeks.

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